

العمر(تاريخ الميلاد) : 04-12-1954 المهنة : بدون عمل

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه
التوقيع: (-----)

التعليق على التشخيص : -----

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 جهة العلاج :
 ملاحظات :
 نفقات :

رئيس
المجلس الطبي المتخصص

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Name: عزيزه عبدالمنعم محمد البكري National ID: 25412040101587	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Anthracycline-based chemotherapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جوستاف
روني
مركز
مصر

بأذن
9-14



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الصحة المصرية
مصر

09/13/2026

MDT FORM

Name: عزيزه عبدالمنعم محمد البلكي	MRN: 13363
Age: 72 Y	ID: 25412040101587
Menopausal Sta: POST MENOPASUAL	Phone: * 01206763060
NRS SCORE: 0	Marital Status: MARRIED
BMI:	Family History: FREE
Gender:	Reason Of MDT: NEW
CO Morbidity: DIABETIC ON ORAL HYPOGLYCEMICS	ECOG Performance Status © 0 1 2 3 4
Surgical History: * HERNIOPLASTY	

Clinical Data:

BX: INVASIVE DUCT CARCINOMA
ER: 8/8 PR: 5/8 HER2: 1+ KI67: 25%

History:

Sonomammography:

UOQ MASS LESION 7X 6X 6 CM
MULTIPLE MORE THAN 3 AXILLARY LYMPH NODES

Biomarker/Molecular

Mets Workup:

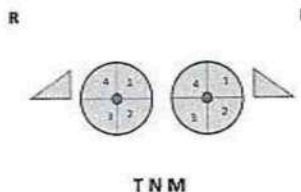
CAP PULMONARY NODULES 7 MM
AFTER REVISION OF RADIOLOGY DEPARTEMENT (FOR FOLLOW UP)

Recommendations:

FUNGATING BREAST MASS FOR POSSIBILITY OF NACT THEN RT MRM

MDT Outcome & Plan

PORTACATH , ECHO
NACT(AC X4 , TAXOTERE X4) ,
EVALUATION US EVERY 2 CYCLES
THEN RT MRM



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د محمد كيلاني	د محمد المنار اوي		

جمهورية مصر العربية
بطاقة تعريف شخصية

عزیزہ
عبد المنعم محمد البلكی
٦٤ ش - ميدان النافوره
المقطم - القاهره

٢٥٤١٢٠٤٠١٠١٥٨٧

IH0557279

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٧/١٦ عین

٢٠١٩/١٢ ٢٥٤١٢٠٤٠١٠١٥٨٧
ریة منزل

أنثى مسلمة متزوجة
صالح زينهم صالح حسن
البطاقة سارية حتى ٢٠٢٦/١٢/٢٣

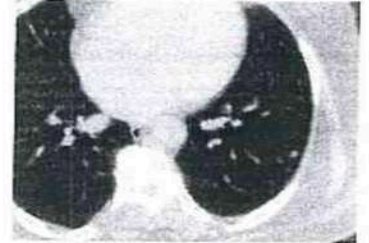
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Patient's Name: Aziza Abdul-Moneim Mohammed Al-Balaki
Date: 2026-08-25
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Pre and post triphasic Multi-Slice CT scan of the Chest and Abdomen Revealed:

Chest findings

- The scanned part of right breast revealed large malignant looking breast lesion with diffuse breast skin thickening associated with multilevel right axillary suspicious LNs, the largest measuring about (3.8 x2.5xm)
- Few bilateral pulmonary nodules seen at both lung fields, the largest seen at upper segment of left lower lung lobe measuring about (7.5 cm) ...? **Metastatic for follow up.**
- Patent trachea, main and segmental bronchi.
- No significant enlarged hilar or mediastinal lymphadenopathy.
- Clear pleural surfaces with no evidence of pleural effusion.
- Normal cardiac size. No pericardial effusion.
- Bilateral elastofibroma dorsi (incidental findings).



Abdomen and pelvis findings

- **Enlarged liver.** No abnormal attenuation values or enhancing lesions.
- No dilated intra-hepatic biliary radicles.
- Normal caliber patent Portal Vein
- Normal gallbladder. No hyperdense gall stones. Normal extrahepatic bile ducts.
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- Normal aorta, I.V.C.
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- Degenerative changes of the scanned bones with dorso-lumbar scoliosis its concavity to the right side.
- Small sclerotic bone island noted at right femoral neck.
- No aggressive bony lesions.

CONCLUSION:

- Known case of right breast cancer with bilateral suspicious pulmonary nodules as described ...for follow up.
- Other findings as described.
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PROF/HANAN EL-AHWAL

Patient Name	عزيزه عبدالمنعم محمد	Case No.	342461
Sex/Age	Female 71 Y	Patient No.	24886
Registered Date	16/7/2026 02:00:53 PM	Referred By Dr.	Out Patient
Validation Date	05/8/2026 09:53:06 AM	Branch	Gustave Roussy
Reporting Date	18/8/2026 11:18:42 AM	Pathology No.	2246/26

Clinical Data: Right breast UOQ mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 16/07/2026.

Gross Examination:

- Five grey, white tissue cores, the longest is 1 cm, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- **Histologic type:** Invasive duct carcinoma.
- **Histologic grade:** 2 (differentiation 3, pleomorphism 2, mitosis 2).
- **Ductal carcinoma in situ:** Absent.
- **Angioinvasion:** Present.
- **Perineural invasion:** Absent.

Results:

*** IPT (All controls showed appropriate reactivity):**

- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Moderate nuclear reaction in 30% of tumor cells (score 5/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells



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(score 1+).

- Ki67%: 25%.

Conclusion:

Right breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 2.

IPT:

- ER: Positive (score 8/8).

- PR: Positive (score 5/8).

- Her2: Low (score 1+).

- Ki67%: 25%.

**Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

** Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

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and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
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جوسناف
رونيست
مركز سرطان الثديمبادرة رئيس الجمهورية
لحماية صحة المرأة المصريةالجمهورية العربية السورية
مدير

09/13/2026

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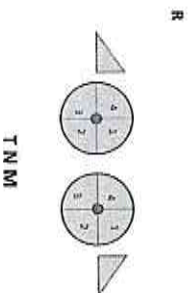
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د محمد كبري	د محمد الطائي		



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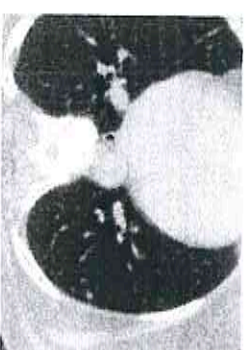
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